

PROPOSALPULSE

Gold Team Review

RFP Response Assessment

DHA Electronic Health Record Modernization

RFP Response Draft

March 2026

PREPARED FOR

Sample Evaluation

PREPARED BY

ProposalPulse by Mission Meets Tech

Federal Health IT Intelligence

Federal health IT intelligence. Mission first.

01 Scorecard Summary

74

/100

OVERALL SCORE

74 / 100

COMPETITIVE — NEEDS WORK

DHA EHR Modernization | March 2026 | 9 criteria evaluated

1.	Compliance & Responsiveness	B		82
2.	Technical Approach	C+		76
3.	Management Approach	B-		78
4.	Past Performance	A-		88
5.	Staffing & Key Personnel	C		72
6.	Cost/Price Realism	B		80
7.	Innovation & Value-Add	D+		65
8.	Risk Mitigation	C+		74
9.	Writing Quality & Clarity	C		70

02 Priority Fix List

Top 3 actions to maximize competitiveness. Ordered by impact on evaluation score.

1 Strengthen Innovation Section

Innovation & Value-Add | **D+** → **B** | +12 pts

The value-add narrative is generic. Reference specific DHA modernization pilots (e.g., MHS GENESIS Wave 4 deployment at Madigan Army Medical Center) and quantify efficiency gains. Current section lacks concrete metrics — add at least 3 quantified outcomes from analogous past work.

2 Sharpen Key Personnel Bios

Staffing & Key Personnel | **C** → **B+** | +10 pts

Bios read like resumes. Rewrite each to map the individual's experience directly to PWS requirements. Lead with the specific contract task they'll own, then cite 2-3 relevant accomplishments with measurable results. Current bios average 400 words — target 250 with higher signal density.

3 Improve Writing Concision

Writing Quality & Clarity | **C** → **B** | +8 pts

Several sections exceed recommended page limits. Tighten the executive summary to 1 page (currently 2.5). Reduce passive voice by 40% — the technical volume has 62% passive construction rate. Eliminate redundant capability statements that appear in both the management and technical volumes.

PROJECTED pWin IMPROVEMENT

38% → 52%

with recommended fixes | +14 pts | Projected score: 74 → 104 weighted points

Methodology: pWin derived from AI-assisted analysis against RFP evaluation criteria, historical award patterns, and proposal quality signals. For illustrative purposes only.

03 Detailed Criteria Analysis

Compliance & Responsiveness

B | 82

STRENGTHS

Strong compliance matrix. All L/M requirements addressed with clear traceability. Cross-reference table is well-organized and evaluator-friendly.

WEAKNESSES

Minor gap: didn't address evaluation subfactor 3.2.1 on data migration timeline. Missing two PWS reference callouts in Section 4.

RECOMMENDATIONS

Add explicit response to subfactor 3.2.1 with a 90-day migration timeline. Insert PWS reference callouts in Sections 4.3 and 4.7.

Technical Approach

C+ | 76

STRENGTHS

Solid architecture overview with clear microservices decomposition. Good use of DoD reference architecture frameworks. Security approach is comprehensive.

WEAKNESSES

Lacks specificity on MHS GENESIS compatibility and interoperability. No references to Wave 4 lessons learned. Missing integration architecture diagram.

RECOMMENDATIONS

Add integration architecture diagram showing MHS GENESIS data flows. Reference specific Wave 4 lessons learned from Madigan AMC. Include FHIR API endpoint mapping.

Past Performance

A- | 88

STRENGTHS

Strongest section. Three relevant CPARS with Exceptional/Very Good ratings. All references are recent (within 3 years) and directly relevant to DHA health IT.

WEAKNESSES

Narratives focus on scope rather than outcomes. No quantified performance metrics (e.g., uptime, user adoption rates, cost savings).

RECOMMENDATIONS

Add quantified outcomes from each reference: uptime percentages, user adoption rates, cost savings achieved. Target 3 metrics per reference.

Innovation & Value-Add

D+ | 65

STRENGTHS

Mentions relevant technology trends (AI/ML, cloud migration, zero trust). Shows awareness of DHA modernization goals.

WEAKNESSES

Weakest section. Generic technology buzzwords without DHA-specific context. No concrete pilot references or measurable outcomes. Reads as marketing material rather than technical substance.

RECOMMENDATIONS

Complete rewrite. Reference 3 validated pilot programs with measurable outcomes. Quantify efficiency gains (e.g., '40% reduction in clinician documentation time'). Tie each innovation to a specific PWS requirement.

04 Evaluator Risk Flags

Issues likely to trigger evaluator concerns or downgrade.

HIGH Key Personnel

Program Manager Bio Lacks DHA Experience

Program Manager bio does not demonstrate DHA-specific experience. Evaluators weight PM qualifications heavily under Factor 3. Without direct DHA program references, this will likely score below competitive range.

HIGH Innovation & Value-Add

Innovation Section Reads as Boilerplate

Without specific pilot references, evaluators may score this non-competitive. Generic technology buzzwords (AI, cloud, zero trust) without DHA context signal a lack of domain expertise. This is the single highest-risk section.

MEDIUM Writing Quality

Executive Summary Exceeds Length

Executive summary exceeds 2 pages (currently 2.5). Evaluators typically stop reading after page 1. Key discriminators buried on page 2 may never be seen. Tighten to 1 page with a clear win theme statement.

MEDIUM Cost/Price Realism

Cost Narrative Missing Basis of Estimate

Cost narrative lacks basis of estimate for 3 labor categories (Sr. Systems Engineer, Data Architect, Clinical Informaticist). May trigger a cost realism concern and request for clarification, adding evaluation risk.

LOW Compliance

Undefined Acronyms

Two acronyms used without definition on first use: FHIR (Fast Healthcare Interoperability Resources) and HL7 (Health Level Seven). Minor issue but signals lack of attention to detail and may frustrate evaluators unfamiliar with health IT standards.

05 Competitive Positioning

Where this proposal stands relative to typical winning submissions.

CRITERIA	YOUR SCORE	WIN THRESHOLD	GAP
Compliance & Responsiveness	82	80	+2
Technical Approach	76	82	-6
Management Approach	78	80	-2
Past Performance	88	85	+3
Staffing & Key Personnel	72	78	-6
Cost/Price Realism	80	78	+2
Innovation & Value-Add	65	80	-15
Risk Mitigation	74	76	-2
Writing Quality & Clarity	70	78	-8



KEY INSIGHT

Your Past Performance (88) and Compliance (82) are at or above win threshold. Innovation (65) and Writing Quality (70) are the primary gaps. Addressing these two areas alone projects a +14 point score increase.

06 What Happens Next

YOUR PROPOSALPULSE ASSESSMENT INCLUDES TWO DELIVERABLES:

1

This Assessment Report

Scoring, analysis, risk flags, and prioritized recommendations to maximize your competitiveness.

2

Polished Final Version

A strengthened rewrite of your original document with highlighted improvements and confidence scores on each changed section.

METHODOLOGY

Scores derived from AI-assisted analysis of proposal text against RFP evaluation criteria, Section M weighting, and historical award patterns for DHA health IT contracts. pWin model incorporates incumbent advantage, team composition, pricing competitiveness, and proposal quality signals. For illustrative purposes only — actual evaluations may vary.

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